

Your GLP-1 Support Kit

A free printable companion to
Your Body's Own GLP-1
by Rev. Dr. LaVeena B. Archers

The tools from the book, gathered in one place you can print, mark up, and bring with you. Keep it on the refrigerator, in your bag, or beside you at your next appointment. Everything here is education and support, not medical advice.

The GLP-1-Supporting Plate

A simple template for meals that raise and steady your own GLP-1. Build every plate in this order.

- Protein first: eggs, fish, poultry, plain Greek yogurt, beans, lentils, tofu, or lean meat, at every meal and especially at breakfast.
- Fiber-rich plants next, filling much of the plate: a wide, changing range of vegetables, plus beans, lentils, oats, barley, and berries.
- Good fats: olive oil, nuts, seeds, avocado, and fatty fish, rather than heavy saturated fats.
- Fast, refined carbohydrates kept to the edges rather than the center.
- Eat slowly, protein and vegetables before starch, and give your fullness signals time to arrive before a second helping.

Protein Cheat Sheet

Because protein is the single hardest thing to get enough of on a small appetite, lean on this quick list of easy, protein-rich foods, and try to include one at every meal.

- Eggs and egg whites.
- Plain Greek yogurt, skyr, and cottage cheese.
- Fish and seafood, including canned tuna and salmon.
- Poultry and lean meats.
- Beans, lentils, chickpeas, and edamame, which bring fiber along with the protein.
- Tofu, tempeh, and other soy foods.
- A simple protein powder stirred into yogurt, oats, or a smoothie, for when whole foods fall short.

Measures That Matter (not only the scale)

Track a few of these over months, alongside or instead of the scale. Movement in the right direction is how you know the work is working, often long before the number does.

- ☐ Waist measured at the navel
- ☐ Strength, such as how many sit-to-stands you can do, or what you can lift
- ☐ Energy and sleep quality, noticed across a whole week
- ☐ Blood pressure, blood sugar (A1C), and cholesterol, tracked with your provider
- ☐ How your clothes fit and how you move through an ordinary day

Questions to Bring to Your Prescriber

Print this, check the ones that apply to you, and take it into the room.

- ☐ Given my health and history, do the benefits of one of these medicines outweigh the risks for me specifically?
- ☐ How will we protect my muscle and bone as I lose weight, and should I have a bone-density scan first?
- ☐ What side effects should I expect, and how will we manage them?
- ☐ What is the plan if I want to stop one day, and what usually happens then?
- ☐ What will this cost, for how long, and what does my insurance cover?

Which Path Fits You Right Now?

I built this for the woman sitting at her kitchen table at eleven at night, wondering whether to call her doctor about a GLP-1 medicine or keep working the plan she already has. This quiz will not diagnose you. It will not prescribe anything for you. Only your own prescriber can do that, after they know your full history, your labs, and your goals. What this page can do is help you take an honest inventory of where you stand right now, so that whatever conversation comes next, with yourself or with your doctor, starts from clarity. Grab a pen. Answer each question the way your life looks today, not the way you wish it looked.

Each answer has a point value in parentheses. Circle the answer that fits you best for each of the ten questions, jot its points in the margin, then add all ten numbers into one total at the bottom of the page. There are no wrong answers here. This is your own private snapshot, not a medical test. The score bands below have no clinical validation behind their cutoffs; they are only a way to organize your own thinking before a conversation with a prescriber.

- 1. How much excess weight are you carrying, and what has it meant for your health so far? a) I am within about fifteen pounds of a healthy range for my body, with no other risk factors (0 points). b) I am carrying fifteen to forty extra pounds, no diagnosis yet, but my labs or blood pressure are drifting the wrong way (2 points). c) I am carrying more than forty extra pounds, and my doctor has already told me this is a genuine risk to my health (4 points). d) I already carry a diagnosis tied directly to this weight, such as type 2 diabetes, heart disease, or severe sleep apnea (6 points).
- 2. Which of these has an actual doctor diagnosed in you? Choose the single tier that fits best. a) None of these (0 points). b) Prediabetes, high blood pressure, or mild fatty liver (2 points). c) One serious diagnosis, such as type 2 diabetes, heart disease, or moderate to severe sleep apnea (5 points). d) More than one of these serious diagnoses at the same time (7 points).
- 3. How long, and how seriously, have you worked the foundations, meaning real food, protein, strength training, sleep, and stress care? a) Less than two months, or I have not really started yet (0 points). b) Two months to six months (1 point). c) Six months to a year, tracked and adjusted along the way (2 points). d) More than a year, including working with a practitioner (4 points).
- 4. What have the foundations done for you so far? a) I have not given them a full, honest try yet, so it is too early to say (0 points). b) I am seeing steady, real progress, and I believe more time will keep helping (0 points). c) I saw progress at first, then plateaued below where my health needs me for more than six months despite staying consistent (3 points). d) I have put in a full, honest year or more and see little to no meaningful change (5 points).
- 5. Could you sustain the cost of a GLP-1 medicine, financially and logistically, for the years it may take? a) No, this is not realistic for my budget or my life right now (0 points). b) It would be a stretch, and possible for a year or two (1 point). c) Yes, I can sustain this for the long haul if needed (2 points).
- 6. How ready are you to protect your muscle and bone during this process, whichever path you choose, meaning protein at each meal and regular strength work? a) I am not currently doing this and do not have a plan yet (0 points). b) I am working on it, but inconsistent (1 point). c) I already prioritize protein and strength training regularly (2 points).
- 7. Which is closest to your real reason for considering any of this? a) Mostly how I look, or pressure from someone else, whether family, a doctor, or a culture that prizes thinness (0 points). b) A mix

of appearance and real health concern (1 point). c) Squarely a real health risk I want to reduce (3 points).

- 8. How do you feel about a GLP-1 medicine as a tool, apart from your current numbers? a) I would rather not take a prescription medicine unless there is truly no other path (0 points). b) I am open to it if my numbers say I need it (1 point). c) I see it as a legitimate tool I would use without hesitation if my health called for it (2 points).
- 9. What is your access to a prescriber who could partner with you on this? a) I do not currently have a doctor I trust to prescribe and monitor this (0 points). b) I have access to one, and we have not discussed it (1 point). c) I already have a prescriber who knows my full history and is willing to partner with me (2 points).
- 10. How is your daily life going right now, meaning your energy, mobility, breath, and sleep? a) It feels fine day to day (0 points). b) I am noticeably limited some days, whether by joints, breath, or energy (2 points). c) My daily life is seriously affected, and I feel my health sliding (4 points).

Total score zero to nine. Your body is not currently telling you it needs a medicine to be safe. The foundations, meaning real food, protein, strength training, sleep, and stress care, are exactly the right focus for you right now, and they will serve you whether or not you ever add a GLP-1 medicine later. Keep building for a few more months, then take this quiz again. If your health picture ever changes, whether a new diagnosis, a new risk, or a plateau despite real effort, bring this page to your prescriber and talk it through with them.

Total score ten to eighteen. The foundations are still your home base, and there is real work left to do there before a medicine decision makes sense. Give yourself an honest, tracked stretch of effort on protein, strength training, sleep, and stress if you have not already, somewhere between six months and a year. Protecting your muscle and bone matters here as much as the number on the scale. When you reach that point, if a real health risk is still sitting in front of you, bring this page to your prescriber and ask what they see in your labs and your history.

Total score nineteen to twenty-seven. You have a real case worth taking to your prescriber. Between the weight involved, any diagnosis already on your chart, the real time you may have already put into the foundations, and your own values about using every legitimate tool available, a GLP-1 medicine deserves a serious look from someone who knows your full health picture. Keep your protein and strength training in place while you have that conversation, because a medicine works better, and protects your muscle and bone better, when the foundations are already underneath it. Bring this page with you to the appointment.

Total score twenty-eight to thirty-seven. Your health is asking for a direct, prompt conversation with your prescriber about a GLP-1 medicine. Between a real diagnosis, a weight carrying genuine risk, years already spent on the foundations, and your own readiness to take this on, the case for at least discussing medicine seriously is strong. Only your prescriber can weigh your full history and decide what is safe and right for you. While you make that call together, keep the foundations in place, meaning your protein, your strength training, and your sleep, because they are what will carry you well through years of using a medicine, coming off one, or moving between the two.

However your numbers land, this quiz is a mirror. It cannot diagnose you, and it cannot write you a prescription. Only a licensed prescriber who knows your full history can make that call with you. A low score does not rule out a real medical need, and a high one does not by itself mean a medicine is right for you. What it can do is help you walk into that conversation, or into another season of building your foundations, with your eyes open and your reasons your own.

Myth vs. Reality: The Supplement Truths

Myth: berberine is nature's Ozempic and does the same job as the drugs, without the risk.

Reality: in a handful of studies berberine produced modest weight loss, on the order of a few pounds, a fraction of what the medicines produce, from studies that are small and mixed. It can also interact with prescription medicines, so clear it with your prescriber first.

Myth: if GLP-1 is your own hormone, you can buy it in a bottle and take it.

Reality: an enzyme in your blood called DPP-4 begins snipping GLP-1 apart the moment your gut releases it, and within a minute or two most of it is gone. Swallowed or injected in its natural form, it would be broken down long before it could do much good.

Myth: my metabolism is broken and my gut has stopped making the hormone that tells me I am full.

Reality: when a large review pooled the studies, people with type 2 diabetes did not, as a group, secrete less GLP-1, and the obesity studies are mixed rather than damning. For most of us that little sensing factory is still running.

Myth: Ozempic and Wegovy are two different medicines, and Mounjaro is the gentle, natural alternative. **Reality:** Ozempic and Wegovy are the same molecule, semaglutide, sold under two names for two purposes. Mounjaro is tirzepatide, itself a weekly injection and one of the most powerful drugs in this family.

Myth: if these medicines protect the heart and the kidneys, everyone should be on them. **Reality:** those findings came from people already carrying significant weight or real disease. A benefit is only a benefit against a danger you face.

Myth: Berberine is nature's Ozempic.

Reality: It is a real compound with modest effects on weight, blood sugar, and cholesterol in mostly small studies, and the weight change is a small fraction of what the medicines produce. The nickname is marketing, not medicine.

Myth: Cold plunges boost your metabolism enough to take weight off.

Reality: Cold really does raise how much you burn in the short term, but the controlled human trials do not show it changing weight on its own.

Myth: Spicy food reliably raises your GLP-1.

Reality: When researchers controlled for the discomfort capsaicin causes, the extra fullness appeared without any matching rise in GLP-1. Enjoy the chili, expect no hormonal favor from it.

Myth: A probiotic capsule will reset your gut and your appetite.

Reality: A few specific strains have raised GLP-1 in small studies, but there is no good evidence that a random probiotic off the shelf changes your weight. Feeding the bacteria you already have is cheaper and better studied.

Myth: Leaky gut is behind your fatigue, anxiety, and stubborn weight, and a supplement stack will seal it.

Reality: The gut barrier is real and can become more permeable in diagnosed conditions like celiac

and inflammatory bowel disease, but the supplements marketed for leaky gut have not held up when tested.

Myth: your BMI is the verdict on your health.

Reality: that formula was built in the 1830s to describe patterns across large populations, not to judge any individual body, and it only became medicine's default generations later, when it was renamed the body mass index in 1972.

Myth: a bigger weight-loss number means the problem is solved.

Reality: a more powerful drug still leaves open whether you can afford it for years, still tends to give the weight back when you stop, and still takes muscle and bone along with fat when the loss is fast.

If this kit helps you, the full story is in *Your Body's Own GLP-1*. And if the book served you, an honest review is how another woman finds it.

This kit is education and support, not medical advice. Every decision about a medicine belongs with you and your own prescriber.